

AI-Powered HMS Synthetic Clinical Dataset

Clinical Note

Patient Name	Phan Thanh Phu	MRN	MRN-0041
DOB	1962-02-21	Gender	Female
Encounter Date	2024-11-28	Department	Oncology
Attending Provider	Pharm. Riya Patel	Status	active

Synthetic Data Warning: This document is generated for software testing, OCR parsing, chunking, embedding, access-control validation, and Graph RAG demonstrations. It is not valid medical advice and must not be used for patient care.

Subjective

Phan Thanh Phu is a 64-year-old patient in Oncology presenting for follow-up of fatigue and treatment-related symptom burden. PMH includes chemotherapy exposure, fatigue, anemia risk, neutropenia precautions reviewed. Patient reports symptoms are stable overall. Denies severe chest pain, syncope, active bleeding, or new focal neurologic deficit unless otherwise documented. Current meds include ondansetron 4mg Q8H PRN; pantoprazole 40mg daily; ferrous sulfate 325mg daily. Allergies: penicillin allergy - rash.

Objective

Vitals: BP 116/81 mmHg, HR 63 bpm, RR 15/min, Temp 37.0 C, SpO2 98% RA, weight 96 kg. Exam: alert, oriented, no acute distress. Cardiopulmonary exam without unstable findings. Abdomen soft, non-tender. Extremities show no acute deformity; edema varies by department-specific condition. Labs reviewed below; medication list reconciled.

Recent Lab Snapshot

Date	Analyte	Value	Unit	Reference Range	Status
2026-05-01	Creatinine	1.25	mg/dL	0.7-1.3	Normal
2026-05-01	eGFR	65.0	mL/min/1.73m2	>=60	Normal
2026-05-01	Glucose	129.0	mg/dL	70-99	High
2026-05-01	HbA1c	5.7	%	<5.7	High
2026-05-01	BNP	268.0	pg/mL	<100	High
2026-05-01	AST	46.0	U/L	10-40	High
2026-05-01	ALT	52.0	U/L	7-56	Normal
2026-05-01	Potassium	4.4	mmol/L	3.5-5.0	Normal
2026-05-01	Hemoglobin	11.3	g/dL	12.0-17.5	Low

Assessment

Primary assessment: active malignancy on treatment with anemia and infection risk monitoring. Patient is clinically stable for ongoing management. Risk considerations include renal dosing, medication safety, fall risk, and need for timely escalation if red-flag symptoms occur.

Plan

Trend CBC, AST, ALT, and renal function. Assess fever, mucositis, and hydration at each contact. Coordinate oncology follow-up and treatment schedule. Provide patient education, update care team via SBAR, and document any access-relevant clinical justification before AI-assisted retrieval of PHI.

Detailed Care Coordination Notes

Problem List

1. active malignancy on treatment with anemia and infection risk monitoring. 2. Medication safety monitoring. 3. Functional status and discharge readiness. 4. Follow-up coordination with Oncology.

Safety Triggers

Escalate to responsible clinician for SBP < 90, HR > 130, SpO2 < 92%, acute mental status change, active bleeding, severe pain, fever with neutropenia risk, or rapidly rising Cr/K.

AI Retrieval Tags

access_tags: read, summary, labs, medication, cardiology as applicable. Suggested document_type for ingestion: clinical_note.